

PERSONALIZED HEALTH ANALYSIS

Adam Hernandez

2026-08-09 · Consultant: Johnny / Irene · Recheck in 4 Weeks

Elevated Visceral Fat at Age 19

Gut / Constipation — Fiber Deficit

Sedentary Pattern + Occasional Binge Eating

CLIENT OVERVIEW

NAME

Adam Hernandez

AGE

19

GENDER

Male

REFERRED BY

(to confirm)

BODY COMPOSITION

WEIGHT

199 lb

Above ideal for height

HEIGHT

178 cm

5'10"

BMI

28.0

Overweight (25–29.9)

BODY FAT

22%

Borderline high (ideal <20%)

VISCERAL FAT

11

Elevated (>10 = concern)

METABOLIC AGE

22

3 yrs above actual (manageable)

BMR

1,889 kcal

Resting baseline

BONE MASS

7.8 kg

Good

Adam's most notable number is visceral fat at 11 — this is elevated for a 19-year-old. Visceral fat (the fat around internal organs, not just under the skin) is metabolically active, producing low-grade inflammation. At 19, elevated visceral fat is an early warning that usually responds very well to dietary fiber and movement changes. His BMR of 1,889 means he has a good metabolic engine. The stool data (dry/cracked, 4+ wipes) and high-meat/low-vegetable diet are directly connected to the visceral fat reading.

TOP 3 PRIORITY SIGNALS

1 Elevated Visceral Fat at Age 19

Visceral fat at 11 (threshold is 10) combined with a high-meat, low-vegetable diet and occasional binge eating is an early metabolic flag at an age where intervention has the highest long-term payoff. This is not alarming — it is an opportunity to prevent a pattern from becoming a chronic condition in his 20s.

2 Gut / Constipation — Fiber Deficit

Stool consistency is dry and cracked (Bristol Type 1–2, indicating dehydration and very low fiber) requiring 4+ wipes — a clear sign that the large intestine is not receiving enough soluble and insoluble fiber to form healthy transit. A diet high in meat and low in vegetables will produce exactly this picture. This also creates a gut microbiome imbalance that can drive acne, mental fog, and occasional blood sugar instability.

3 Sedentary Pattern + Occasional Binge Eating

Prolonged screen-sitting marked Often combined with occasional binge eating creates a caloric surplus without the metabolic outlet of movement. At 19 with a high BMR, Adam has a significant natural advantage — the visceral fat and body fat percentages are still in the manageable range. Establishing movement and hunger-regulation habits now will compound significantly.

ROOT CAUSE ANALYSIS

• Fiber Deficit (Primary Driver)

High-meat, low-vegetable diets produce predictable outcomes: dry/hard stool, gut microbiome imbalance (the bacteria that produce anti-inflammatory short-chain fatty acids are fed by plant fiber), and reduced satiety between meals. The dry/cracked stool (Bristol Type 1–2) requiring 4+ wipes confirms that dietary fiber is severely insufficient. Without fiber, gut transit slows, toxins recirculate through the colon longer, and the liver handles a higher detoxification load. This also means the gut bacteria that regulate appetite and blood sugar are under-supported, which may explain the occasional binge eating — hunger cues become less reliable without fiber slowing glucose absorption.

• Visceral Fat / Inflammation (Secondary Driver)

Visceral fat at 11 in a 19-year-old reflects early metabolic imbalance. Unlike subcutaneous fat (under the skin), visceral fat surrounds organs and is hormonally active — it secretes interleukins and TNF-alpha that create a low-grade inflammatory state. This type of inflammation is directly linked to acne (Adam reports occasional facial acne) and can subtly suppress motivation and focus over time. The high-meat, low-plant diet also contributes omega-6 fatty acids without the omega-3 counterbalance, tipping the prostaglandin balance toward inflammation. This is highly reversible at 19 with consistent dietary adjustment.

• Sedentary Lifestyle

Prolonged sitting marked Often at 19 means the large muscle groups that are the primary consumers of blood glucose are inactive for most of the day. Muscle tissue is the body's main glucose sink — when it is inactive, blood glucose clears more slowly after meals, and the excess is more likely to be stored as visceral fat. Adam's high BMR gives him a natural metabolic advantage that is being partially offset by inactivity. 20–30 minutes of movement daily — any kind — would engage those muscles and meaningfully reduce visceral fat accumulation over a 4-week cycle.

SUMMARY

Adam is in an excellent position — he is 19, has a strong BMR, good sleep, and a manageable baseline. The two things that need to shift are fiber intake (currently near zero from vegetables) and daily movement. The visceral fat and dry-stool picture are early flags, not a crisis, and they respond quickly to these two changes. Adding omega-3 support addresses the inflammatory underpinning of both the acne and the visceral fat in a targeted way.

30-DAY PLAN

1 Vegetables at Every Meal

- Minimum one serving of vegetables or legumes at every meal — non-negotiable for gut health.
- Practical: add a handful of spinach to eggs or a protein shake in the morning, a side salad at lunch, cooked vegetables at dinner.
- Target: 4–5 servings/day by week 4. Start at 2 servings and build.
- This is the single most impactful change for his stool consistency, visceral fat, and acne.

2 Movement — Engage the Metabolic Engine

- 20–30 minutes of any intentional movement daily — walk, gym, basketball, anything.
- Break up screen time: stand or walk 5 minutes every hour.
- Muscle activation improves blood glucose clearance and directly reduces visceral fat accumulation.

3 Structured Eating — Reduce Binge Eating

- Eat protein and fat at regular intervals — binge eating is typically driven by under-eating earlier in the day.
- He already eats breakfast with protein (good). Make lunch consistent too.
- A fiber source at each meal (from food or supplement) will significantly stabilize appetite.

SUPPORT TOOLS (OPTIONAL)

Food, water, sleep, and movement are always the foundation. These are targeted bridge tools for specific gaps.

Advanced Omega

Adam's high-meat, low-plant diet loads the body with omega-6 fatty acids without their omega-3 counterbalance — this tips the body toward a pro-inflammatory state, which shows up as elevated visceral fat, facial acne, and mild cardiovascular risk markers. Advanced Omega provides the EPA and DHA needed to restore this balance. EPA specifically reduces the inflammatory prostaglandins driving visceral fat accumulation; DHA supports the brain function and focus he reports as occasionally reduced.

Take 1–2 softgels with dinner. Effects on visceral fat and acne build over 4–8 weeks of consistency.

Prebiotic Fiber

Adam's stool (dry/cracked, 4+ wipes) is a textbook fiber-deficit picture. Prebiotic fiber provides the soluble fiber that feeds beneficial gut bacteria, regulates transit speed, binds excess cholesterol, and stabilizes blood glucose between meals — directly addressing the binge-eating pattern driven by blood sugar swings. It is also one of the most reliable tools for reducing visceral fat over time when combined with dietary changes.

Mix one serving in water once daily, ideally before a main meal. Stool consistency improvement is typically noticeable within 5–7 days.

WHAT TO EXPECT IN 30 DAYS

- ✓ Improved stool consistency (less dry/cracked) within 5–7 days as fiber intake increases
- ✓ Reduced appetite spikes and binge eating urges within 1–2 weeks as fiber stabilizes blood glucose
- ✓ Reduction in facial acne at 4–6 weeks as gut-skin axis improves and omega-3 balance shifts
- ✓ Visceral fat trending toward 10 or below at the 4-week recheck with consistent fiber and movement
- ✓ More consistent energy and mental clarity as gut microbiome diversifies

NEXT STEP

OPTION A — LIFESTYLE ONLY

Vegetables at every meal (4+ servings/day), 30-min movement daily, structured eating schedule. Recheck visceral fat, body fat %, and stool consistency at 4 weeks.

OPTION B — LIFESTYLE + SUPPORT

Advanced Omega at dinner + Prebiotic Fiber before lunch, plus all lifestyle changes above. Recheck visceral fat, stool consistency, acne at 4 weeks.

4 WEEKS RECHECK

Visceral fat level, body fat %, stool consistency (Bristol scale), acne frequency, energy level (1–5)

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